

Background

- Medications for rare diseases require close clinical monitoring and care coordination.
- Manufacturers may require risk evaluation and mitigation strategies (REMS) for medication management.
- Integrated health system specialty pharmacies (HSSP) are ideally positioned to meet the complex requirements of managing patients with rare diseases.
- In February 2025, the integrated HSSP at Yale New Haven Health achieved the Utilization Review Accreditation Commission's (URAC) Rare Disease Certification.
- Mavacamten and trientine, penicillamine are specialty medications approved for treating obstructive hypertrophic cardiomyopathy and Wilson disease, both rare diseases.

Objective

- To describe the impact of the integrated HSSP on turnaround time (TAT), medication assistance, and pharmacist clinical services for rare diseases.

Methods

This is a retrospective chart review of patients who received prescriptions for mavacamten, and trientine, penicillamine from January 1, 2024, to December 30, 2024, for cardiomyopathy and Wilson disease, respectively.

Dispenses were categorized as requiring intervention if clinical or insurance issues were encountered during prescription processing or were otherwise categorized as clean.

The primary outcome included evaluating specialty clinical pharmacist services for managing rare diseases.

Secondary outcomes included evaluating TAT and financial assistance.

Results

Table 1: Demographic Data

Cardiomyopathy	
Total patients	65
Male	52%
Female	48%
Average age (yrs)	66

Figure 1: Turnaround time (business days) for Mavacamten

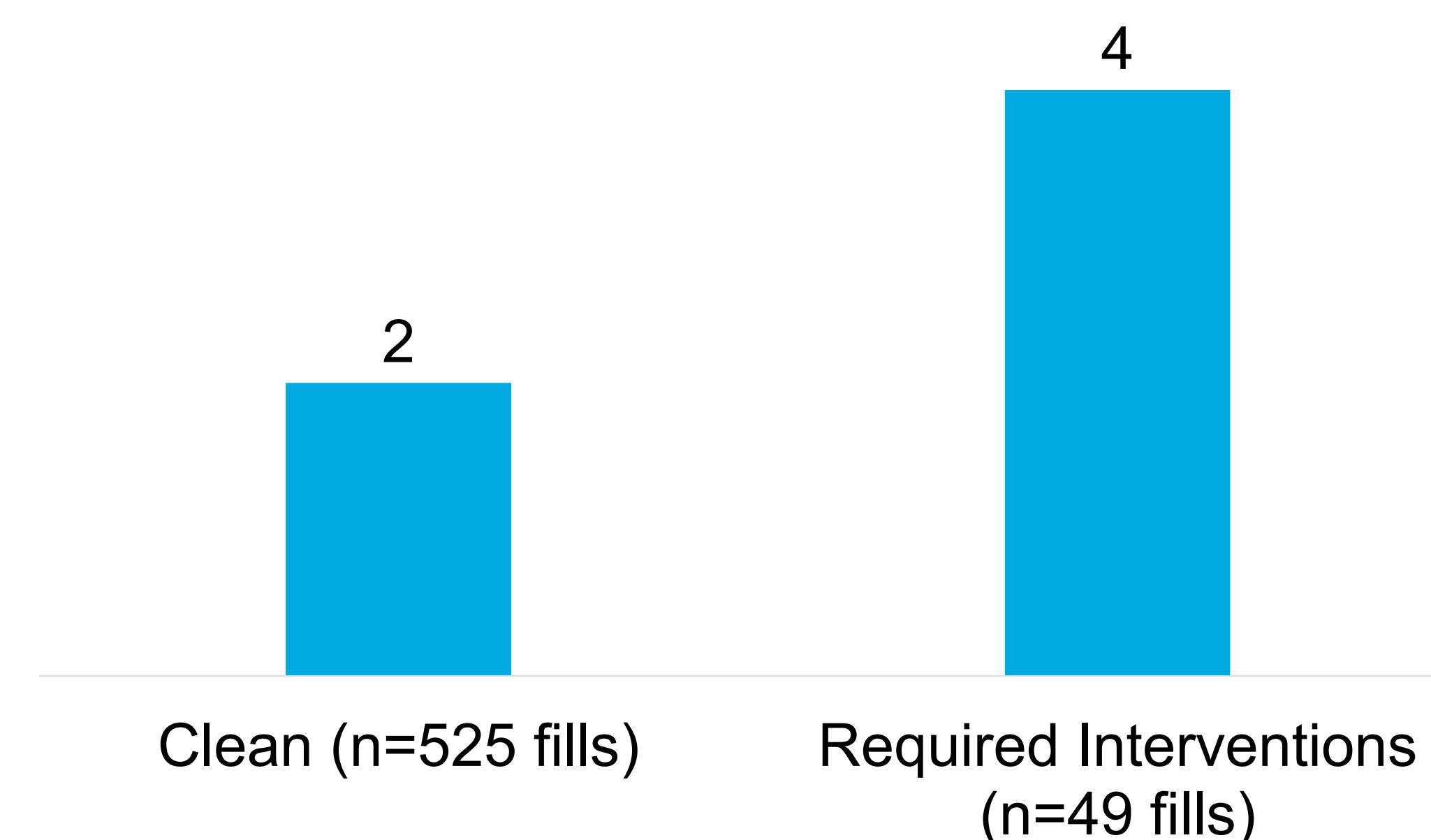
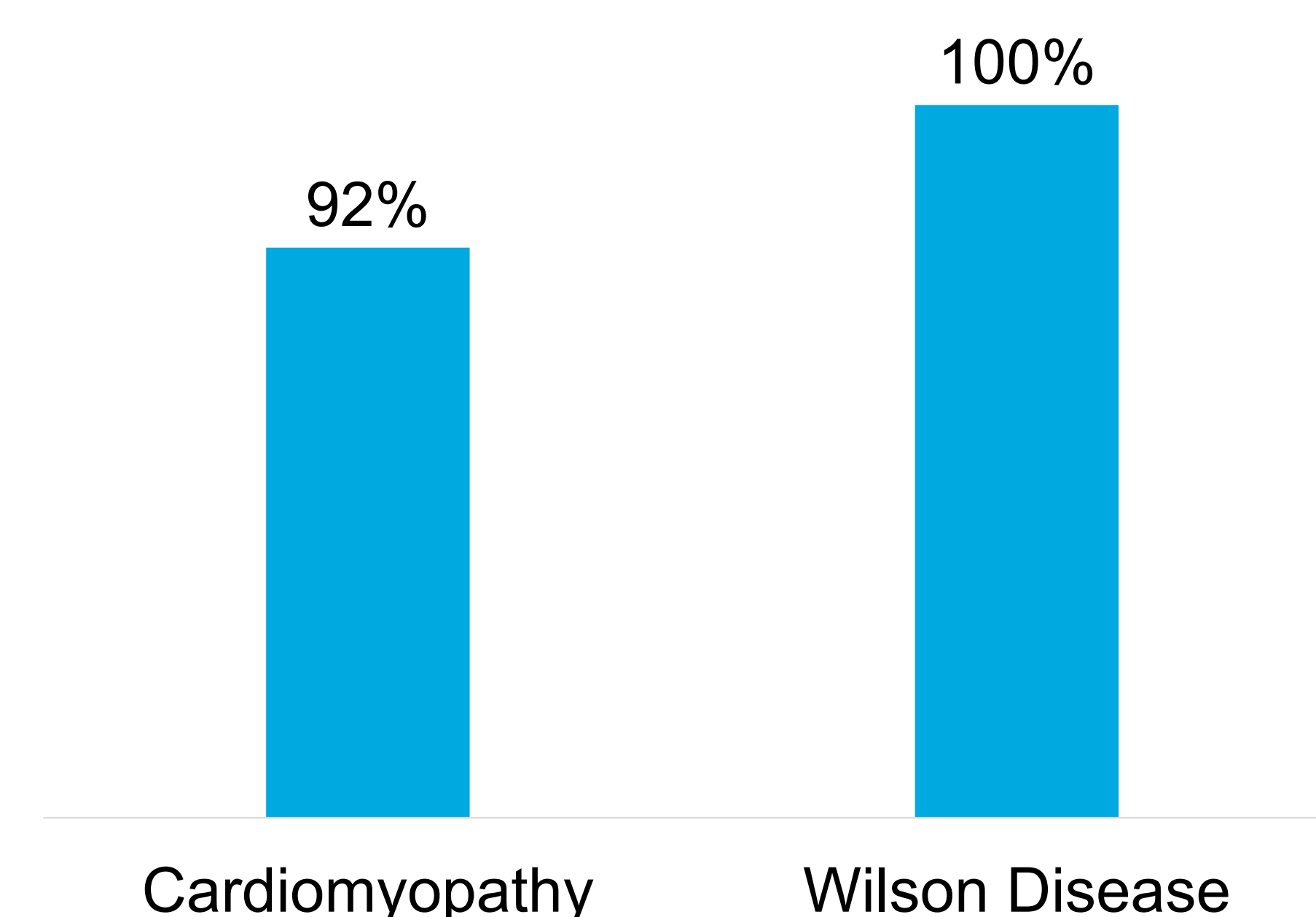


Figure 3: Patients staying on therapy as of May 2025



Wilson's Disease	
Total patients	51
Male	51%
Female	49%
Average age (yrs)	39

Figure 2: Turnaround time (business days) for Penicillamine, Trientine

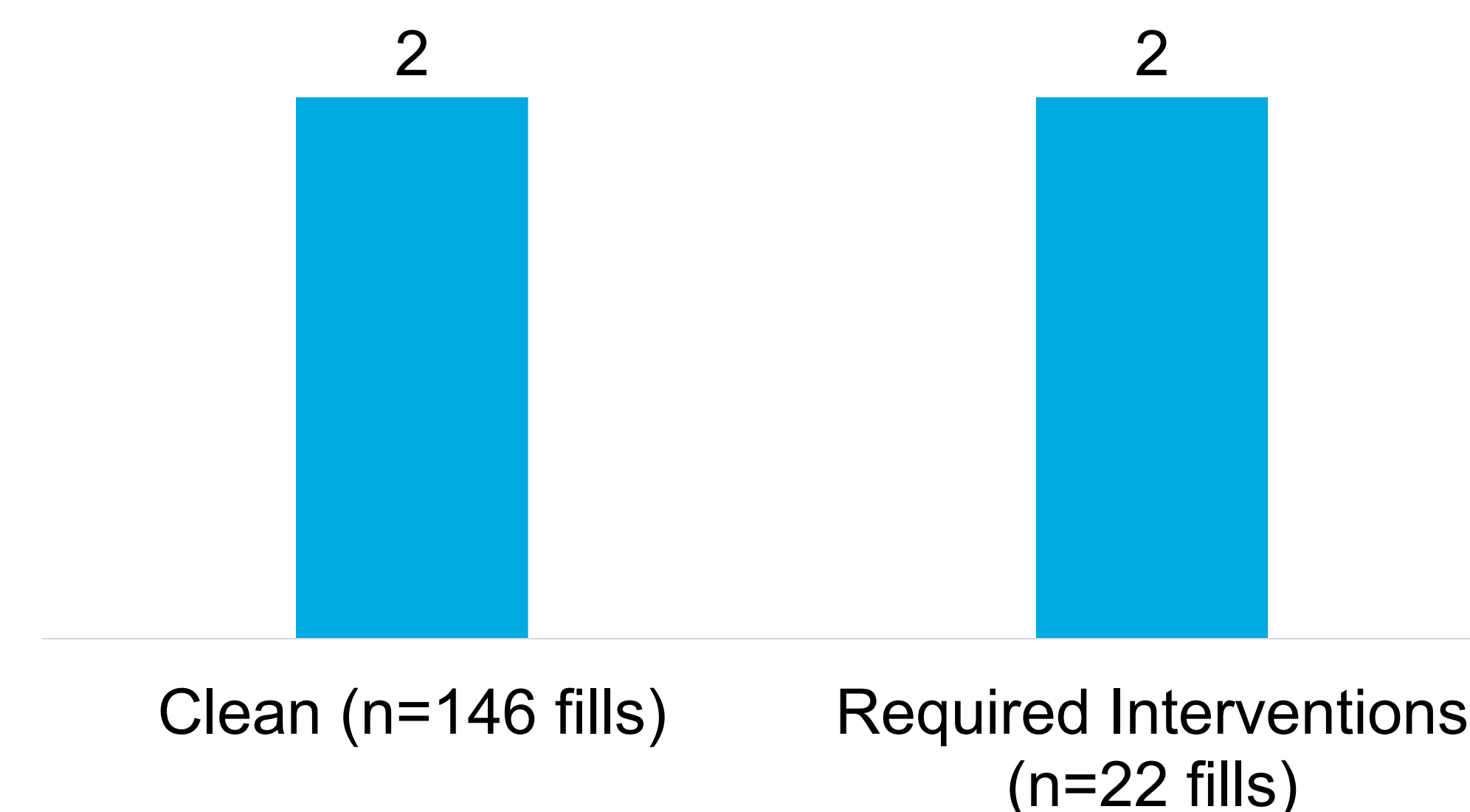


Table 2: Specialty pharmacy services for rare diseases (n= 116 patients)

Clinical assessments	311
Medication therapy interventions	73
Deliveries for mavacamten	517

\$ Financial Assistance: 26 patients

Discussion

- At the integrated HSSP, specialty pharmacy liaisons assist with applying for prior authorizations, handling appeals, and processing prescriptions leading to expedited treatment initiation.
- Complex payor models, specialty drug carve outs, deductibles etc. may hinder the access of specialty medications for patients with rare diseases.
- About 22% of patients enrolled in the cardiomyopathy or Wilson disease received financial assistance at the integrated HSSP.
- Early initiation of therapy also improves treatment and patient reported outcomes¹
- High touch services provided at the integrated HSSP such as clinical assessments, medication therapy interventions, and routine outreach for refills ensured patients stayed on therapy.

Limitations

- Data elements required to calculate TAT are not standardized across specialty pharmacies resulting in differences in TAT definitions.²
- Health systems might not have access to the medications used to treat rare diseases, as most are Limited Distribution Drugs (LDD).

Future Directions

- Patients with rare diseases need easy access to treatment and resources. Allowing integrated HSSPs access to LDD medications will improve outcomes for patients.

Conclusion

Integrated HSSP's high touch model is suited for navigating rare disease patient management.

References

- Bisni Narayanan, Michele Riccardi, Terri Sue Rubino, Tina Do. Evaluation of Mavacamten Clinical Outcomes at an Integrated Health System Specialty Pharmacy One Year Post Implementation. Presented at NASP Annual Meeting 2025, Denver, CO.
- [Piloting the PQA Specialty Pharmacy Turnaround Time Quality Measure](#), accessed 07/02/25